

Why Regular Market Forces Do Not Reduce Low-Value Care: A Logistics and Systems Perspective

Answer

From a logistics and systems perspective, Low-Value Care (LVC) is not only a clinical problem but a failure of feedback, incentives, and resource allocation across a complex service network. In a normal market, inefficient services are expected to decline because customers compare price and quality, suppliers compete for demand, and inefficient producers lose revenue. However, this logic assumes that market signals are allowed to operate. In healthcare, those signals are often weakened or replaced by government budgets, regulated reimbursement, professional licensing, public procurement rules, insurance design, and administrative planning. Therefore, LVC should not be understood simply as a failure of markets; it is better understood as a consequence of a system where ordinary market forces are only partially present.

The European Commission's expert report defines LVC broadly as overuse, misuse, underuse, and unwarranted variation in healthcare services [1]. VALUE-SHIFT similarly treats LVC as a persistent system-level inefficiency across care pathways, not merely as a collection of isolated bad decisions [2]. This distinction is important. In logistics, waste often persists not because actors want waste, but because the system routes resources through distorted incentives. A hospital department, a diagnostic unit, an insurer, a public purchaser, and a patient may each respond rationally to their own local incentives while the whole pathway produces duplicated tests, unnecessary follow-ups, avoidable referrals, or care in a more expensive setting than necessary.

The first reason ordinary market forces do not eliminate LVC is that prices are often hidden from the patient and separated from the act of consumption. In most healthcare systems, patients do not pay the full marginal cost of each visit, scan, procedure, or follow-up. Payment is mediated through taxation, social insurance, private insurance, or public reimbursement. Pauly's classic analysis of moral hazard shows that insurance changes behaviour because the consumer is partly insulated from the price of care [3]. The RAND Health Insurance Experiment also found that medical use responds to out-of-pocket prices, showing that healthcare demand is not completely price-insensitive [4]. If the patient experiences an additional test or appointment as nearly free at the point of use, the demand-side pressure to reject low-value activity is weak.

The second reason is information asymmetry. Patients usually cannot judge whether a diagnostic test, specialist referral, or follow-up schedule is clinically necessary. Arrow's analysis of medical care emphasises uncertainty and the unusual information structure of healthcare markets [5]. In ordinary consumer markets, a buyer can often compare alternatives directly. In healthcare, the provider frequently acts both as advisor and supplier. This is not a moral accusation against clinicians; it is a structural problem. When the buyer cannot easily evaluate value, and when the price is also hidden, the ordinary consumer discipline that removes low-value services becomes weak.

The third reason is that provider incentives often reward activity rather than value. In logistics, a system produces what it is paid and measured to produce. If reimbursement is attached to visits, admissions, procedures, or diagnostic activity, then reducing low-value activity may reduce revenue or disturb established staffing and budget patterns. VALUE-SHIFT explicitly identifies misaligned payment mechanisms as a barrier, especially where activity-based financing rewards volume over value [2]. From a market-oriented perspective, this is not genuine market competition. It is an administered payment structure that can make low-value activity financially rational even when it is clinically or socially inefficient.

The fourth reason is fragmentation of the care pathway. LVC often emerges across interfaces: between primary care and hospitals, between diagnostics and treatment, between specialist clinics and community services, or between acute and long-term care. VALUE-SHIFT describes LVC as arising through duplication, misaligned follow-up, unwarranted variation, fragmented multimorbidity management, and disproportionate service intensity [2]. In logistics terms, this resembles a supply chain in which each unit optimises locally while the total flow is inefficient. One department may reduce its own risk by referring the patient onward; another may repeat diagnostics because records are incomplete; a hospital may keep follow-up in a high-cost setting because the existing pathway, staffing model, or reimbursement code is built that way.

The fifth reason is the “wrong-pocket” problem. A provider that reduces LVC may not capture the benefit. For example, fewer hospital follow-ups may save specialist time but increase pressure on primary care. Shorter hospital stays may free beds but require more community services. Fewer diagnostic scans may save system resources but reduce income for the diagnostic unit. OECD distinguishes between wasteful clinical care, operational waste, and governance-related waste, which is useful because it shows that healthcare waste is often produced by the design of the system, not only by individual clinical choices [6]. Where costs and benefits fall into different budgets, there is no clear residual claimant with a strong incentive to remove low-value activity.

The sixth reason is that regulation and licensing restrict adaptive capacity. In an ordinary market, new entrants, substitute services, and alternative production models can challenge high-cost incumbents. In healthcare, entry and substitution are limited by professional licensing, scope-of-practice rules, accreditation, public procurement, liability rules, and reimbursement eligibility. Kleiner’s work on occupational licensing shows that licensing can restrict entry and reduce competition, with price and access effects [7]. Stigler’s theory of regulation also warns that regulation can be shaped by organised interests and may protect incumbent groups rather than only consumers [8]. Applied to healthcare logistics, this means that services which could potentially be delivered by lower-cost providers, digital tools, nurse-led models, or community settings may remain locked into expensive structures. Safety regulation may be necessary, but it still slows the market process of experimentation, substitution, and exit.

The seventh reason is that capacity is rigid. Healthcare resources are not easily redeployed. Hospital beds, specialist time, diagnostic equipment, and regulated professional roles are fixed in the short run. Even when low-value activity is identified, eliminating it does not automatically create cash savings. It may only release time that is absorbed by waiting lists, emergency demand, or unmet need elsewhere. VALUE-SHIFT’s attention to workforce utilisation, released capacity, and redeployment reflects precisely this logistical problem [2]. In a normal market, resources tend to move toward higher-value uses through price and profit signals. In healthcare, movement is mediated by budgets, contracts, staffing rules, and political cycles.

Finally, market forces require measurement and transparency, but LVC is often difficult to observe. The European Commission report identifies data access, data quality, lack of suitable indicators,

and methodological uncertainty as major obstacles in measuring LVC [1]. If patients cannot compare outcomes and prices, providers cannot benchmark pathways, and payers cannot identify avoidable activity accurately, then inefficient routines can persist inside normal practice. Hayek's argument about the price system is relevant here: markets coordinate dispersed knowledge through prices and local decisions [9]. Healthcare often suppresses that coordination mechanism and replaces it with administrative rules that are slower and less sensitive to local information.

In conclusion, regular market forces do not reduce LVC because the institutional conditions for markets are not fully present in healthcare. Prices are obscured, patients are insulated from marginal costs, providers may be rewarded for volume, information is asymmetric, savings are split across budgets, capacity is rigid, and regulation or licensing limits competition and substitution. From a logistics and systems perspective, LVC persists because healthcare is governed less like a competitive market and more like a regulated, third-party-paid, administratively planned service network. The relevant point is not that efficiency is impossible in healthcare, but that efficiency does not emerge automatically when the normal mechanisms of market correction are blocked.

References

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